United Methodist Homes
Skilled Nursing Facility
Policy and Procedure

SUBJECT: RESIDENT RIGHTS

REGULATION:

OBRA: F550-555, 557-580, 582-586, 559, 600, 602-605, 745

NEW YORK STATE: §415.3, 415.5

PENNSYLVANIA: §201.20, 201.29

POLICY STATEMENT:

The United Methodist Homes recognizes each resident's rights to a dignified existence, self-determination, and communication with and access to persons and services inside and outside the facility. It is the policy of United Methodist Homes to treat each resident with respect and dignity, recognize each resident's individuality, care for each resident in a manner and environment that promotes maintenance or enhancement of his or her quality of life, and to protect, promote, and respect such rights guaranteed to residents under Federal and State laws and regulations.

DEFINITIONS:

"As soon as is reasonably possible"- In Pennsylvania, this shall be defined as 30 days prior to the change unless circumstances dictate otherwise.

"Beneficiary notices"- CMS notices completed and issued by the facility to communicate resident financial liability and appeal rights under the Fee-for-Service (FFS) Medicare and the Medicare Advantage (MA) Programs. Examples include the:

a. Notice of Medicare Non-Coverage; and
b. Skilled Nursing Facility Advanced Beneficiary Notice of Non-coverage.

"Items and services that may be charged to residents' funds"- General categories and examples of items and services that the facility may charge to residents' funds if they are requested by a resident, if they are not required to achieve the goals stated in the resident's care plan, if the facility informs the resident that there will be a charge, and if payment is not made by Medicare or Medicaid:

a. Telephone, including a cellular phone;
b. Television/radio, personal computer or other electronic device for personal use;
c. Personal comfort items, including notions and novelties, and confections;
d. Cosmetic and grooming items and services in excess of those for which payment is made under Medicaid or Medicare;
e. Personal clothing;
f. Personal reading matter;
g. Gifts purchased on behalf of a resident;
h. Flowers and plants;
i. Cost to participate in social events and entertainment outside the scope of the activities program;

j. Non-covered special care services such as privately hired nurses or aides;
k. Private room, except when therapeutically required (for example, isolation for infection control); and
l. Specially prepared or alternative food requested (but not medically prescribed) instead of the food and meals generally prepared by the facility and take in consideration the overall cultural and religious make-up of the facility's population.

"Items and services that may not be charged to residents' funds"- Categories of items and services for which payment is made under Medicaid or Medicare (except for applicable deductible and coinsurance amounts) that may not be charged to residents' funds:

a. Nursing services as required by regulation;
b. Food and Nutrition services as required by regulation;
c. An activities program as required by regulation;
d. Room/bed maintenance services;
e. Routine personal hygiene items and services as required to meet the needs of residents, including, but not limited to:
    i. hair hygiene supplies including a comb and brush;
    ii. bath soap;
    iii. disinfecting soaps or specialized cleansing agents when indicated to treat special skin problems or to fight infection;
    iv. razor and shaving cream;
    v. oral hygiene products including a toothbrush, toothpaste, denture adhesive, denture cleaner, and dental floss;
    vi. moisturizing lotion;
    vii. tissues;
    viii. cotton balls and cotton swabs;
    ix. deodorant;
    x. incontinence care and supplies;
    xi. sanitary napkins and related supplies;
    xii. towels and washcloths;
    xiii. hospital gowns;
    xiv. over the counter drugs;
    xv. hair and nail hygiene services;
    xvi. bathing assistance; and
    xvii. basic personal laundry;
f. Medically-related social services as required by regulation; and
g. Hospice services elected by the resident and paid for under the Medicare Hospice Benefit or paid for by Medicaid under a state plan.

"Resident rights"- Civil and legal rights of nursing home residents. Examples include the resident's right to:

- Equal access to quality care regardless of diagnosis, severity of condition, or payment source;
- Be informed about what rights and responsibilities he or she has and of all rules and regulations governing resident conduct and responsibilities during his or her stay in the facility;
- Exercise his or her rights as a resident of the facility and as a citizen or resident of the United States;
- Be free from abuse, neglect, misappropriation of resident property, and exploitation;
- Have the facility manage his personal funds if he or she wishes;

- Choose a physician (excluding physicians who do not meet or comply with all pertinent regulatory requirements);
- Be informed in advance, by the physician or other practitioner or professional, of the risks and benefits of proposed care, of treatment and treatment alternatives or treatment options and to choose the alternative or option he or she prefers;
- Be informed, in advance, of the care to be furnished and the type of care giver or professional that will furnish care;
- Be informed of, and participate in, his or her treatment and participate in decisions and care planning (including the right to accept, request, refuse, and/or discontinue treatment), to participate in or refuse to participate in experimental research, and to formulate an advance directive (including the right to receive information regarding and formulate advance directives once he or she is able in the event that he or she was incapacitated at the time of admission and unable to receive information or articulate whether or not he or she has executed an advance directive.);
- Confidentiality and personal privacy including accommodations, medical treatment, written and telephone, and email communications , personal care, visits, and meetings of family and resident groups;
- Send and promptly receive unopened mail and other letters, packages and other materials delivered to the facility for the resident, including those delivered through a means other than a postal service;
- Communicate with individuals and entities within and external to the facility, including reasonable access to the internet, and access to stationery, postage, and writing implements (available at the resident's own expense);
- Have reasonable access to and privacy in their use of electronic communications such as email and video communications and for internet research;
- Secure and confidential personal and medical records;
- Receive notices orally (meaning spoken) and in writing (including Braille) in a format and a language he or she understands in accordance with State and Federal law;
- Retain and use personal possessions, including furnishings, and clothing, as space permits, unless to do so would infringe upon the rights or health and safety of other residents and expect the facility to exercise reasonable care for the protection of said property from loss or theft;
- A safe, clean, comfortable and homelike environment, including but not limited to receiving treatment and supports for daily living safely and expect the facility to ensure that he or she can receive care and services safely and that the physical layout of the facility maximizes his or her independence without safety risk;
- Voice grievances and have the facility respond to those grievances;
- Organize and participate in resident groups in the facility and to expect the facility provide private meeting space and to take reasonable steps, with the approval of the group, to make residents and family members aware of upcoming meetings in a timely manner
- Examine the results of the most recent survey of the facility conducted by Federal or State surveyors and any plan of correction in effect with respect to the facility;
- Work or not work, including the right to choose or refuse to perform services for the facility;
- Visit and be visited by others from outside the facility (subject to reasonable clinical and safety restrictions and the resident’s right to deny or withdraw consent at any time);
- Have reasonable access to the use of a telephone, including TTY and TDD services, and the ability to use a telephone, including the resident’s personal cellular phone, a place in the facility where calls can be made without being overheard;
- Be informed of changes in resident rights under Federal or State law;

- Access and be provided with personal and medical records pertaining to him or herself in a form and manner the resident can access and understand;\
- Remain in the facility, and not be transferred or discharged from the facility unless the reason(s) for the discharge are consistent with applicable federal and state guidelines;
    - Supplemented in Pennsylvania by the right to be informed as to whether the facility where the resident is being transferred is certified to participate in the Medicare and the Medical Assistance Programs;
- Be notified, along with his/her representative (if any), of changes and have his/her physician consulted as indicated when there is an accident involving the resident which results in injury and has the potential for requiring physician intervention, a significant change in the resident's physical, mental, or psychosocial status, a need to alter treatment significantly, and/or a decision to transfer or discharge the resident from the facility due to a condition or situation that would endanger the health or safety of the resident or other individuals in the facility;
- Manage his or her financial affairs, including the right to know, in advance, what charges the facility may impose against a resident's personal funds;
- Be told in advance when changes will occur in their bills, including but not limited to issuance of federally required forms such as beneficiary notices;
- Not be required to deposit his or her personal funds with the facility but, when choosing to deposit personal funds with the facility, expect the facility to act as a fiduciary of the funds and hold, safeguard, manage, and account for the deposited personal funds;
- Have refunded to the appropriate individual all refunds due within 30 days from the date of discharge from the facility, including the remaining balance of any deposit or charges already paid after deduction of the facility's per diem rate for the days the resident actually resided or reserved/retained a bed in the facility, regardless of any minimum stay or discharge notice requirements;
- Receive written notice, including the reason for the change, before the resident's room or roommate in the facility is changed;
- Retain and use personal possessions to the maximum extent that space and safety permit;
- Share a room with a spouse, if that is mutually agreeable;
- Share a room with a roommate of choice when practicable, and if that is mutually agreeable;
- Self-administer medication, if the interdisciplinary care planning team determines it is safe;
- Refuse to transfer to another room in the facility, if the purpose of the transfer is to:
    - relocate a resident of a SNF from the distinct part of the institution that is a SNF to a part of the institution that is not a SNF; or
    - to relocate a resident of a NF from the distinct part of the institution that is a NF to a distinct part of the institution that is a SNF; or
    - solely for the convenience of staff;
- Be free of interference, coercion, discrimination, and reprisal from the facility in exercising his or her rights;
- Have the facility record and periodically update the address (mailing and email) and phone number of the resident representative(s);
- Have a same-sex spouse afforded treatment equal to that afforded to an opposite-sex spouse if the marriage was valid in the jurisdiction in which it was celebrated;
- Be supported by the facility in the exercise of his or her rights;
- Receive information from agencies acting as client advocates, and be afforded the opportunity to contact these agencies; and

- In the case of a resident who has not been adjudged incompetent by the state court, designate a representative, in accordance with State law, enabling the legal surrogate so designated to exercise the resident's rights to the extent provided by state law.

"Exercising of rights": Residents have autonomy and choice, to the maximum extent possible, about how they wish to live their everyday lives and receive care, subject to the facility's rules, as long as those rules do not violate a regulatory requirement.

"Local Contact Agency" [New York State definition]- An agency designated by the Department to accept referrals of nursing home residents that wish to receive information about services in the community.

"Nursing home staff (staff)"- Staff includes employees, consultants, contractors, volunteers, and other caregivers who provide care and services to residents on behalf of the facility.

"Notice of Medicare Non-Coverage (NOMNC)"- Form CMS-10123, issued by the facility to all Medicare beneficiaries at least two days before the end of a Medicare covered Part A stay or when all of Part B therapies are ending, which informs the beneficiary of the right to an expedited review by a Quality Improvement Organization.
[Note: The NOMNC is not given if:

a. The beneficiary exhausts the SNF benefits coverage (100 days), thus exhausting their Medicare Part A SNF benefit;
b. The beneficiary initiates the discharge from the SNF; or
c. The beneficiary elects the hospice benefit or decides to revoke the hospice benefit and return to standard Medicare coverage.]

"Reasonable access"- The ability of the resident to access the internet for communications and internet research when to following criteria is met:

a. access is available to the facility; and
b. the resident uses the internet in compliance with State and Federal law, including all legal limitations and restrictions imposed thereto.

[Note: Access may be at the individual resident's expense, if any additional expense is incurred by the facility to provide such access to the resident(s).]

"Resident representative"- For purposes of this policy, the term may mean any of the following:

a. an individual chosen by the resident to act on behalf of the resident in order to support the resident in decision-making; access medical, social or other personal information of the resident; manage financial matters; or receive notifications;
b. a person authorized by State or Federal law (including but not limited to agents under power of attorney, representative payees, and other fiduciaries) to act on behalf of the resident in order to support the resident in decision-making; access medical, social or other personal information of the resident; manage financial matters; or receive notifications;
c. a legal representative, as used in section 712 of the Older Americans Act; or
d. the court-appointed guardian or conservator of a resident.

"Right to personal privacy": The resident's right to meet or communicate with whomever they want without being watched or overheard. Private space may be created flexibly and need not be dedicated solely for visitation purposes.

"Skilled Nursing Facility Advanced Beneficiary Notice of Non-coverage (SNF ABN)"- Form CMS-10055, issued only if a Medicare beneficiary intends to continue services in the facility, notifies the beneficiary that the SNF believes the services may not be covered under Medicare, along with the option to continue services with the beneficiary accepting financial liability for those services.

PROCEDURE:

Staff Training Related to Resident Rights

1. Provide initial, on-going yearly, and as needed training and education (including related policy expectations) for nursing home staff related to:
   a. rights guaranteed to residents under Federal and State law;
   b. the expectation that all staff protect, promote, and respect such rights;
   c. reporting responsibilities and facility procedures should staff identify, suspect, or be made aware of an allegation of the violation of a resident's rights;
   d. potential consequences related to the failure to protect, promote, and respect such rights including, but not limited to:
      i. disciplinary action up to and including termination (for employees);
      ii. reporting of such violations/failure(s) to the primary employer and potential prohibition of future utilization of the staff member's services at the United Methodist Homes (for agency/contract employees); and
      iii. reporting of such violations/failure(s) to State and Federal agencies in accordance with all pertinent laws and regulations.

[Note: For contract agency staff, the provision of training in accordance with facility policy may be assigned to the contract agency.]
[Note: For volunteers, training shall occur prior to the beginning of volunteer services and as needed thereafter.]
RESPONSIBILITY: Director of Staff Development or Designee

Facility Response to Allegations of Violations of Resident's Rights

1. Report any potential violation(s) of the rights of resident(s) in the facility to supervisory, administrative, or other staff, such as the Grievance Official, designated to receive such reports by the facility.
RESPONSIBILITY: All Staff

2. Anytime an allegation of rights violations are reported, immediately initiate and conduct a thorough investigation, implement corrective actions as indicated to prohibit further violations, and report the findings (preliminary and final) to all regulatory agencies as required.
[Note: Follow the protocols established in UMH abuse-related investigation and reporting policies when applicable.]
RESPONSIBILITY: Administration, Grievance Official, or Designee

3. In the event of an alleged violation of resident's rights, take immediate action as necessary to prevent further potential violations while the allegation is being investigated.
RESPONSIBILITY: Administrator, Grievance Official, or Designee

4. Immediately report all alleged violations involving neglect, abuse, including injuries of unknown source, and/or misappropriation of resident property, by anyone furnishing services on behalf of the facility, to the administrator, and to other agencies as required by Federal and State

law.
RESPONSIBILITY: Grievance Official, or Designee

5. If the alleged violation of the residents' rights is confirmed by the facility or if an outside entity having jurisdiction, such as the State Survey Agency, Quality Improvement Organization, or local law enforcement agency confirms a violation for any of these residents' rights within its area of responsibility, take appropriate corrective action in accordance with State law.
RESPONSIBILITY: Administrator, Grievance Official, or Designee

General Guidelines

1. Provide to residents and applicants for admission, oral and written information about how to apply for and use Medicare and Medicaid benefits, and how to receive refunds for previous payments covered by such benefits and display related written information in the facility.
RESPONSIBILITY: Admissions, Social Worker, or Designee

2. At the time of admission to the facility, when the resident becomes eligible for Medicaid, and, when changes are made to the items and services included in the categories listed below, inform each Medicaid-eligible resident, in writing, of:

a. Items and services that are included in nursing facility services under the State plan and for which the resident may not be charged; and
b. Those other items and services that the facility offers and for which the resident may be charged, and the amount of charges for those services.

RESPONSIBILITY: Admissions, Social Worker, or Designee

3. Provide a notice of rights and services to the resident prior to or upon admission and during the resident's stay. This includes:

a. Informing the resident both orally and in writing of his or her rights and all rules and regulations governing resident conduct and responsibilities during the stay in the facility; and
b. Providing the resident with the State-developed notice of Medicaid rights and obligations, if any.

[Note: Receipt of such information, and any amendments to it, by the resident/resident representative must be acknowledged in writing. Retain such acknowledgement in the medical record.]
RESPONSIBILITY: Admissions, Social Worker, or Designee

4. Upon admission, provide the resident with information and contact information, orally and in writing, regarding the filing of grievances or complaints concerning any suspected violation of state or federal nursing facility regulations, including but not limited to resident abuse, neglect, exploitation, misappropriation of resident property in the facility, non-compliance with the advance directives requirements and requests for information regarding returning to the community.
[Note: If the resident is sight-impaired or hearing-impaired or otherwise unable to comprehend English or printed matter, ensure the information is conveyed in a format and a language that the resident understands.]
RESPONSIBILITY: Administrator, Admissions, Social Worker or Designee

5. Notify the resident individually or through postings in prominent locations throughout the facility of:

a. the right to file grievances orally or in writing;
b. the right to file grievances anonymously;

c. the contact information of the grievance official with whom a grievance can be filed;
d. a reasonable expected time frame for completing the review of the grievance;
e. the right to obtain a written decision regarding his or her grievance; and
f. the contact information of independent entities with whom grievances may be filed, that is, the pertinent State agency, Quality Improvement Organization, State Survey Agency and State Long-Term Care Ombudsman program or protection and advocacy system.

RESPONSIBILITY: Administrator, Grievance Official, or Designee

6. Furnish additional notices orally and in writing to the resident including:
   a. A written description of legal rights which includes:
      i. A description of the manner of protecting personal funds;
      ii. A description of the requirements and procedures for establishing eligibility for Medicaid, including the right to request an assessment of resources under section 1924(c) of the Social Security Act;
      iii. A list of names, addresses (mailing and email), and telephone numbers of all pertinent State regulatory and informational agencies, resident advocacy groups such as the State Survey Agency, the State licensure office, the State Long-Term Care Ombudsman program, the protection and advocacy agency, adult protective services where state law provides for jurisdiction in long-term care facilities, the local contact agency for information about returning to the community and the Medicaid Fraud Control Unit; and
      iv. A statement that the resident may file a complaint with the State Survey Agency concerning any suspected violation of state or federal nursing facility regulations, including but not limited to resident abuse, neglect, exploitation, misappropriation of resident property in the facility, non-compliance with the advance directives requirements and requests for information regarding returning to the community;
   b. Contact information for the Aging and Disability Resource Center; and
   c. Information and contact information for filing grievances or complaints concerning any suspected violation of state or federal nursing facility regulations, including but not limited to resident abuse, neglect, exploitation, misappropriation of resident property in the facility, non-compliance with the advance directives requirements and requests for information regarding returning to the community.

RESPONSIBILITY: Admissions, Social Worker, or Designee

7. Inform each resident before, or at the time of admission, and periodically during the resident's stay, of services available in the facility and of charges for those services, including any charges for services not covered under Medicare/ Medicaid or by the facility's per diem rate. This includes:
   a. Where changes in coverage are made to items and services covered by Medicare and/or by the Medicaid State plan, providing notice to residents of the change as soon as is reasonably possible (e.g., using applicable beneficiary notices when required to ensure Medicare beneficiaries are told in advance when changes will occur in their bills and that they are fully informed of related financial liability, appeal rights, and protections); and
   b. Where changes are made to charges for other items and services that the facility offers, the facility must inform the resident in writing at least 60 days prior to implementation of the change.

RESPONSIBILITY: Admissions, Case Manager, Social Worker, Therapist, or Designee

8. When a resident requests an item or service for which a charge will be made, ensure that the resident has been informed, orally and in writing, that there will be a charge for the item or service and what the charge will be.

[If a resident makes a request for an additionally billable item and has not previously been fully informed of the associated fee, provide the resident with required information prior to the provision of the item or service.]
RESPONSIBILITY: All Staff

9. Post the results of the most recent survey of the facility in a place readily accessible to residents, and family members and legal representatives of residents.
RESPONSIBILITY: Administrator or Designee

10. Have reports of facility surveys, certifications, and complaint investigations made during the three preceding years, along with any plan of correction in effect for the facility, available for any individual to review upon request and post notice of document availability in areas of the facility that are prominent and accessible to the public.
[Note: Identifying information about complainants or residents contained in such reports shall not be made available.]
RESPONSIBILITY: Administrator or Designee

11. Post, in a form and manner accessible and understandable to residents/resident representatives:

a. A list of names, addresses (mailing and email), and telephone numbers of all pertinent State agencies and advocacy groups, such as the State Survey Agency, the State licensure office, adult protective services where state law provides for jurisdiction in long-term care facilities, the Office of the State Long-Term Care Ombudsman program, the protection and advocacy network, home and community based service programs, and the Medicaid Fraud Control Unit; and
b. A statement that the resident may file a complaint with the State Survey Agency concerning any suspected violation of state or federal nursing facility regulation, including but not limited to resident abuse, neglect, exploitation, misappropriation of resident property in the facility, and non-compliance with the advanced directives requirements and requests for information regarding returning to the community.

RESPONSIBILITY: Administrator or Designee

12. In New York facilities, conspicuously post contact information for the Local Contact Agency at wheelchair height in a public place in the facility that is frequented by residents and visitors.
RESPONSIBILITY: Administrator or Designee

13. In Pennsylvania, ensure the inclusion of residents, or a resident representative, in the development, implementation, and review of the policies and procedures regarding the rights and responsibilities of residents. Such inclusion should be fostered through facility groups such as resident counsel.
RESPONSIBILITY: Administrator or Designee

14. Promote and support the exercising of rights for each resident, including any who face barriers (such as communication problems, hearing problems and cognition limits) in the exercise of these rights.
[Note: A resident determined to be incompetent should be able to assert these rights based on his or her degree of capability.]
RESPONSIBILITY: All Staff

15. Ensure that the resident can exercise his or her rights without interference, coercion,

discrimination, or reprisal from the facility.
RESPONSIBILITY: All Staff

16. Do not hamper, compel, treat differentially, or retaliate against a resident for exercising his/her rights.
RESPONSIBILITY: All Staff

17. When providing care and services, respect each resident's individuality, as well as honor and value their input.
RESPONSIBILITY: All Staff

18. Interact with residents in a manner that takes into account the physical limitations of the resident, assures communication, and maintains respect.
RESPONSIBILITY: All Staff

19. Observe and respond to non-verbal communication of a resident who is unable to verbalize preferences (i.e., if the resident spits out food, determine if this is considered to be a choice and alternative meal options should be offered).
RESPONSIBILITY: All Staff

20. Provide health information and services in ways that are easy for the resident and/or the resident's representative to understand.
RESPONSIBILITY: All Staff

21. In the case of a resident or court-designated representative, the facility shall:
    a. Support the representative's right to exercise the resident's rights to the extent those rights are delegated to him or her by the resident who has not been adjudged incompetent by the state court or by a court of law in accordance with State law;
    b. Support the resident's right to exercise those rights not delegated to a resident representative, including the right to revoke a delegation of rights, except as limited by State law;
    c. Support the resident's right to make decisions outside the representative's authority when a resident representative's decision-making authority is limited by State law or court appointment;
    d. Support the resident's right to revoke his or her previous delegation of rights to a resident representative when the resident has not been adjudged incompetent by the state court;
    e. Treat the decisions of a resident representative as the decisions of the resident to the extent required by the court or delegated by the resident, in accordance with applicable law;
    f. Not extend to the resident representative the right to make decisions on behalf of the resident beyond the extent required by the court or delegated by the resident, in accordance with applicable law;
    g. To the extent practicable, provide the resident with opportunities to participate in the care planning process;
    h. Ensure the resident's wishes and preferences are be considered in the exercise of rights by the representative;
    i. Report concerns both when and in the manner required under State law if there is reason to believe that a resident representative is making decisions or taking actions that are not in the best interests of a resident; and
    j. Maintain appropriate documentation related to the delegation of a representative in the medical record. For legal representatives:

i. obtain documentation that the resident's representative has been delegated the necessary authority to exercise the resident's rights;
ii. verify that a court-appointed representative has the necessary authority for the decision-making at issue as determined by the court.
iii. make reasonable efforts to ensure access to documentation of any change related to the delegation of rights, including a resident's revocation of delegated rights.

[Note: Seek a health care decision representative only when the resident is unable to make the decision, including when a resident has been declared incompetent by a court. If it is determined that the resident understands the risks, benefits, and alternatives to proposed health care and expresses a preference, then the resident's wishes should be considered to the degree practicable, including resident input into the care planning process.]

RESPONSIBILITY: All Staff

22. In the case of an impending facility closure, in addition to the provision of required notifications to the State Survey Agency and the Office of the State Long-Term Care Ombudsman, the administrator must provide facility residents and their legal representatives with written notification prior to the impending closure to. The notice shall:

a. Be provided at least 60 days prior to the date of closure, or, if the Secretary or a State terminates the facility's participation in the Medicare and/or Medicaid programs, not later than the date that the Secretary determines appropriate;
b. Include the State or Commonwealth-approved plan for the transfer and adequate relocation of facility residents by a date that would be specified by the State prior to closure, including assurances that the residents will be transferred to the most appropriate facility or other setting in terms of quality, services, and location, taking into consideration the needs, choice, and best interests of each resident.

[Note: The facility shall not admit any new residents on or after the date on which such written notification is submitted.]

RESPONSIBILITY: Administrator

23. Refer to the "Resident Access and Visitation Rights" policy for detailed guidance related to facility visitation.

RESPONSIBILITY: All Staff

POLICY AND PROCDURE: RESIDENT RIGHTS

DATE: 11-2016

APPROVED BY:

Brian Piccini — President and CEO

11/11/17 — Date

Martha A Wright MSRN RD LDN — Clinical Services

11·18·16 — Date

Ronald Patti — Chief Operating Officer

02/02/2024 — Date

REVIEWED/REVISED:

Updated:
01/09/17; 11/2017; 02/2020; 06/08/21; 02/01/24